

From hospice to *home health*: what VitalisCare has to change

Sync was built for hospice: steady census, caregivers as the contact, visits in facilities. Home health reverses those assumptions. It runs on short payer-gated episodes, therapy-heavy caseloads and patients who schedule their own days. Below is every change the difference forces, sized by how much of it exists today.

Capacity management	CAN WE TAKE THE PATIENT?	Scheduling	RIGHT VISIT, RIGHT DAY, LEGALLY	Engagement	PATIENT AND CLINICIAN
Demand shape BUILD HOSPICE steady census, long stays. HOME HEALTH 30-day payment periods in 60-day certifications, front-loaded visits, fast turnover. → Forecast demand by where each patient sits in the episode, not by census. Their own slide marks Demand “needs building.”		Authorization BUILD Hospice schedules after admission. In home health, auth is a gate at SOC and a per-visit ceiling set by the payer. → A ready / not-ready state. Hold unauthorized visits off the calendar. Re-check on add-ons and recerts.		Who you talk to BUILD HOSPICE the caregiver, often in a facility. HOME HEALTH the patient, at home, with a day that changes. → A patient-facing layer designed for home health. Nothing exists today; it was deprioritized for hospice.	
The referral decision BUILD Home health accepts or declines at intake. Sync only sees a patient after admission and auth. → A live branch capacity roll-up (on their roadmap) fed straight from our intake platform.		Episode economics BUILD LUPA floor per 30-day period, frequency orders (2w3, 1w4), front-loading. None of this exists in hospice scheduling. → Pace visits against the plan of care; flag LUPA risk and over-utilization. Front-loading logic can be reused.		Day-before confirmation BUILD 30-60 min per clinician per day of calls and texts, often in the evening, across ~10,000 clinicians. → HIPAA-compliant two-way text first, voice later. Taking half of it away is the single largest time return.	
Disciplines & productivity CUSTOMIZE HOSPICE RN, aide, social work, chaplain. HOME HEALTH adds PT, OT, ST and assistants; SOC and OASIS visits weigh more. → Visit-type weights beyond HCHB points; therapy productivity and eval-driven frequency.		Compliance clock BUILD SOC within 48 h, OASIS timepoints, the day 56-60 recert window, MD notice within 48 h of a missed visit. → Replace hospice F2F, HOPE and RN-supervisory alerts with a home health rule set.		Visit reliability BUILD No-shows and late cancels; the 8-9am first visit is the hardest slot and the biggest capacity lever. → On-the-way ETA, patient self-reschedule, and preferences captured before the call.	
Pending-auth demand BUILD Visits waiting on payer auth sit on no calendar and count toward nothing. → Count pending-auth visits as committed demand in every capacity view.		Who owns the week CUSTOMIZE After the 485, our clinicians plan their own week with no scheduler. Sync optimizes through a scheduler. → Route suggestions to the clinician; scheduler by exception only.		Clinician trust CUSTOMIZE Tenured, salaried, territory-based clinicians, versus a GPS-tracked app with route-adherence review. → Assignment rationale shown to the clinician; tracking and review options set by policy.	
Territories BUILD Home health clinicians hold territories for years; hospice caseloads drift with the census. → Territory as a matching rule, plus a branch-director map that follows referral patterns.		Same-day recovery CUSTOMIZE Home health priorities differ: SOC and OASIS visits first, Medicare week Sun-Sat, frequency within the week. → Home health urgency rules in the Call-Off Center, which itself has not shipped (due by year end).		Hard-to-fill visits BUILD Open visits need offers and incentives; Sync has no differential offers. → Surface open visits with offers. Their own slide marks this “needs building.”	
New-clinician ramp CUSTOMIZE New hires are kept off SOC and OASIS visits, not just given a lighter load. → Ramp by visit type as well as volume. Offered in the demo.		Visit plotting & HCHB tasks ROADMAP Frequencies are plotted in HCHB by the evaluating clinician; scheduler workflow tasks pile up. → Plot through write-back and auto-clear HCHB tasks. Promised in “weeks.”		Care-team coordination WORKS AS-IS Multi-discipline coordination with field and office visibility carries over. → Tie it to HCHB so notes reach the team without double entry.	

Risks in asking a hospice vendor to *learn* home health

HIGH Domain fluency PDGM, LUPA, OASIS and recert were absent from their return and demo. Their gap list (auth and patient comms) undercounts the work. Write home health acceptance scenarios before any build.	HIGH “Decision-tree logic” is not simple Payer rules vary, orders change mid-episode, add-ons re-open auth. Treating it as a few rules means brittle logic and slipping dates. Get scope and dates in writing.	HIGH We become their product team Our SMEs specify it and our branches test it. That is real cost and distraction. Design-partner terms with budgeted Compassus time.	HIGH Speed versus compliance safety “Weeks, not months” from a team with 3 live organizations. A missed LUPA or MD notice costs revenue and survey findings. QA, change control and liability terms.
MED No home health proof No reference customer, no measured result, and we would be first and largest by a multiple. We carry the validation; set exit criteria per phase.	MED Competing roadmap Their paying hospice customers shape priorities, and home health work may queue behind them. Tie commitments to milestones.	MED Data freshness Home health same-day recovery needs fresher HCHB data than their hospice clients accept. Our 3-hour feed and screen-based write-back are both fragile. Fix the feed on our side.	MED IP, durability, adoption What we co-design ships to others. Size and funding were not disclosed. GPS tracking lands on salaried clinicians. Lead time, financials and a tracking policy.